



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ليلي معتمد محمد احمد
المحافظة : الجيزة
رقم البطاقة : 26105152501166 رقم التليفون : 01200867865
العنوان : عزبة السلام
العمر (تاريخ الميلاد) : 15-05-1961 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ليلي معتمد محمد احمد National ID: 26105152501166	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 إني
 مفيد محمد احمد
 عزبة السلام
 مركز الصف - الجيزة
 ٢٦١.٥١٥٢٥.١١٦٦
 JC4181050


 ٢٠٢١/١٠ ٢٦١.٥١٥٢٥.١١٦٦
 علمه زراعيه
 إني
 مسلمة
 مفودة
 عبدالمنعم محمد عبدالقوي حسن
 البطاقة سارية حتى ٢٠٢٨/١٠/٢٦



JKL

Dms
 ٢١/١٠/٢٠٢١
 Cms
 Cms



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنيت المصرية
مصر

03/17/2026

MDT FORM

Name:	Lilaa Matamed Mohamoud Mohamoud	MRN:	6311
Age:	65 Y	ID:	26105152501166
Menopausal Sta		Phone:	
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: PR: HER2: KI67:

R

L

Sonomammography:

Mets Workup:

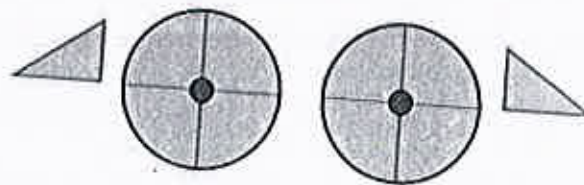
Rt breast cancer PD on multiple lines,
Was thought to be metastatic but MRI proved HFL to be hemangioma.
Received palliative RTh to RT breast.
Patient had bleeding Ulcer.
After surgical evaluation; Patient is Inoperable

History:

Recommendations:

MDT Outcome & Plan

For Oral Navelbine then re-assess for operability after 6 months



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ خالد عبد العزيز	د/ محمد النور				

Patient Name: المرحومة / أم محمد
Patient No. : 12223-2021
Path No. : 12778/24
Sex : Female
Age : 63 Y
Referring Doc. :

Request No. : 13009116
Request Date : 08/08/2024
Verification Date : 18/08/2024
Financial Category : اداءات التأمين - فئة الدولة
Department : Out Patient
IHO No. :
Registered by : marwa mohamed youssou

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Newly developed right lateral chest wall subcutaneous nodule, in follow-up a case of breast cancer.

Nature of Specimen: Tru-cut biopsy, collected at 05/08/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 26% of tumor cells (Allred score 6/8).
- PR: Mild nuclear reaction in 70% of tumor cells (Allred score 6/8).
- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 80% nuclear reaction of tumor cells.

Conclusion:

Right chest wall subcutaneous nodule, Tru-cut biopsy:
Mixed invasive duct & lobular carcinoma, Grade II.

IPT:

- ER: Positive (score 6/8).
- PR: Positive (score 6/8).
- Her2: Negative (score 0).
- Ki67: Positive in 80% of tumor cells.

Verified by :

Printing Date : 20/08/2024 08:45:30

Page 1 of 2

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National Women's Health Initiative
Pathology Lab
مركز البحوث الصحية النسائية



Pathology Report

المرضى: 1299/21
Age: 60 years.
Clinical data: Right breast.
Specimen type: Tru cut biopsy.
Date (Received): 11/6/2021.
Date (reported): 11/7/2021
Lab Number: 1299/21.
Hospital Number: 2069/21

Gross Examination

Procedure: Tru cut biopsy
Localization: Rt breast
Technique: Sections prepared from paraffin blocks for immunostaining
Bench-Mark ULTRA system for ER, PR, Her-2 and Ki67
ER: 100% (100%), PR: 100% (100%), Her-2: 1+ (1+), & Anti-Ki67 (30-40%)

Micronscopic Examination

Histological type: IDC
Grade: II

Remarks

Rt breast mass, tru cut biopsy

In situ duct carcinoma GII

Immunohistochemistry:

- ER: strong intensity, 90% nuclear reaction (Allred score 8/8)
- PR: strong intensity, 90% nuclear reaction (Allred score 8/8)
- Her-2: negative (score 0)
- Ki67: 70% nuclear reaction

Prof. Gamal Abdelaty
Dr. Rana Fahmy



Patient Name : الأستاذة / ليلى محمد محمد
Patient No. : 2223-2021
Path No. : 2778/24
Sex : Female
Age : 63 Y
Referring Doc. :

Request No. : 13009116
Request Date : 06/08/2024
Verification Date : 18/08/2024
Financial Category : ارقام دار السلام - نقابة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Newly developed right lateral chest wall subcutaneous nodule, in follow-up a case of breast cancer.

Nature of Specimen: Tru-cut biopsy, collected at 05/08/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

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- Her2: No membrane reaction in the tumor cells (score 0).
- Ki67: 80% nuclear reaction of tumor cells.

Conclusion:

Right chest wall subcutaneous nodule, Tru-cut biopsy:
Mixed Invasive duct & lobular carcinoma, Grade II.

IPT:

- ER: Positive (score 6/8).
- PR: Positive (score 6/8).
- Her2: Negative (score 0).
- Ki67: Positive in 80% of tumor cells.

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Page 1 of 2

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Patient Name: الأستاذة / الباشي مهند محمد
Patient No.: 2223-2021
Path No.: 2778/24
Sex: Female
Age: 63 Y
Referring Doc:

Request No.: 13009116
Request Date: 06/08/2024
Verification Date: 18/08/2024
Financial Category: Out Patient
Department: اورام دار السلام - تنلة الدرة
Registered by: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: *ER, PR*
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: *Her2*
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is intense and within < 10% of the invasive tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by:

Date: 20/08/2024 08:45:50

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National Women's Health Initiative
Pathology Lab
معمل الباثولوجي المبادرة الوطنية لصحة المرأة



Pathology Report

المرضى: 1299/21
Age: 60 years.
Clinical data: Right breast.
Specimen type: Tru cut biopsy.
Date (Received): 21/6/2021.
Date (reported): 11/7/2021
Lab Number: 1299/21.
Hospital Number: 2069/21

Gross Examination

Procedure: Tru-cut biopsy
Localization: Rt breast
Histotechnology technique: Sections prepared from paraffin blocks for immune-staining
Stains: Bench-Mark ULTRA system for ER, PR, Her-2 and Ki67
IF: ER (1B2), PR clone (SP1), Her-2 clone (4B5), & Anti-Ki-67 (30-9)

Micronscopic examination

Histological type: IDC
Grade II

Remarks

Rt breast mass, tru cut biopsy
Invasive duct carcinoma GII

Immunohistochemistry

ER: strong intensity, 90% nuclear reaction (Allred score 8/8)
PR: strong intensity, 90% nuclear reaction (Allred score 8/8)
Her2: negative (score 0)
Ki67: 70% nuclear reaction

Prof. Gamal AbdelAly
Dr. Rana Fathy



Exam Information	
Patient Name: Ila M	Date of birth: 15/05/1961
Arabic Name: إلى محمد	Patient Gender: F
Patient RIS ID: 340608	Exam Date: 28/10/2023
Patient HIS ID: 123022727	
Written by:	Approved by:
Dr. Hoda Pathy	Dr. Hoda Pathy

^{99m}Tc MDP Bone Scan Report (24/10/2023)

• Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post IV Injection of 20mCi of ^{99m}Tc MDP
Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Rawan Sherif
Nahla Mamoun, MSc

Mush Oblige



**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:-**

- Follow up for a case of breast cancer.
- Compared to last study dated 05/03/2024.

1. Chest:

- Further progressive course as regards the size of the right breast skin attached irregular speculated mass lesion measures about 7 x 9 cm as compared to 7 X 5 cm still seen invading the right pectoral muscle and inseparable from the overlying skin.
- Stable right axillary level I enlarged lymph nodes.
- Newly developed right lateral chest wall subcutaneous nodule measures about 1.3 cm.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Mild cardiomegaly. No pericardial effusion detected.
- Atheromatous calcifications of the aorta, aberrant right subclavian artery (anatomical variant).

2. Abdomen and pelvis:

- The liver is enlarged. Rather stationary size of the right hepatic lobe segment VI subcapsular lesion currently measuring 17 X 42 mm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Still noted right lumbar mesenteric densely calcified lymph node likely representing post granulomatous sequel.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.